MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

QUESTIONS

Q1. A 25-year-old man falls on his shoulder. He cannot abduct his arm beyond 15°, and there is loss of sensation over the "regimental badge" area. What is the most likely injured structure?

a) Radial nerve
b) Axillary nerve
c) Musculocutaneous nerve
d) Suprascapular nerve

Q2. A patient with chorda tympani injury may present with:

a) Hyperacusis
b) Loss of taste from the posterior third of the tongue
c) Decreased salivation from submandibular and sublingual glands
d) Facial muscle paralysis

Q3. A lower motor neuron lesion of the facial nerve typically results in:

a) Paralysis of the lower half of the face only
b) Paralysis of the entire ipsilateral side of the face
c) Contralateral facial muscle weakness
d) Forehead muscle sparing

Q4. Which muscle is supplied by the anterior interosseous branch of the median nerve?

a) Flexor pollicis longus
b) Flexor digitorum superficialis
c) Palmaris longus
d) Pronator teres

Q5. A patient presents with "waiter's tip" deformity after a difficult delivery. Which part of the brachial plexus is injured?

a) Lower trunk
b) Upper trunk
c) Posterior cord
d) Medial cord

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

Q6. A patient squeezes a pimple on the upper lip and later develops fever and signs of intracranial infection. Which structure is most likely involved?

a) Sigmoid sinus
b) Cavernous sinus
c) Transverse sinus
d) Straight sinus

Q7. Injury to tibial nerve causes loss of:

a) Dorsiflexion
b) Plantarflexion
c) Knee extension
d) Hip abduction

Q8. Middle cerebral artery supplies:

a) Medial surface of brain
b) Lateral surface of brain
c) Occipital lobe
d) Brainstem

Q9. A newborn presents with respiratory distress. Imaging shows abdominal contents in the thoracic cavity due to failure of pleuroperitoneal membrane fusion.

What is the most likely diagnosis?

a) Hiatal hernia
b) Bochdalek hernia
c) Morgagni hernia
d) Eventration of diaphragm

Q10. A man fractures the midshaft of his humerus. Later, he cannot extend his wrist. Which nerve is injured?

a) Median nerve
b) Ulnar nerve
c) Radial nerve
d) Axillary nerve

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

Q11. A patient cannot dorsiflex his foot after injury near the fibular neck. Which nerve is affected?
a) Tibial nerve
b) Common peroneal nerve
c) Deep peroneal nerve
d) Sural nerve

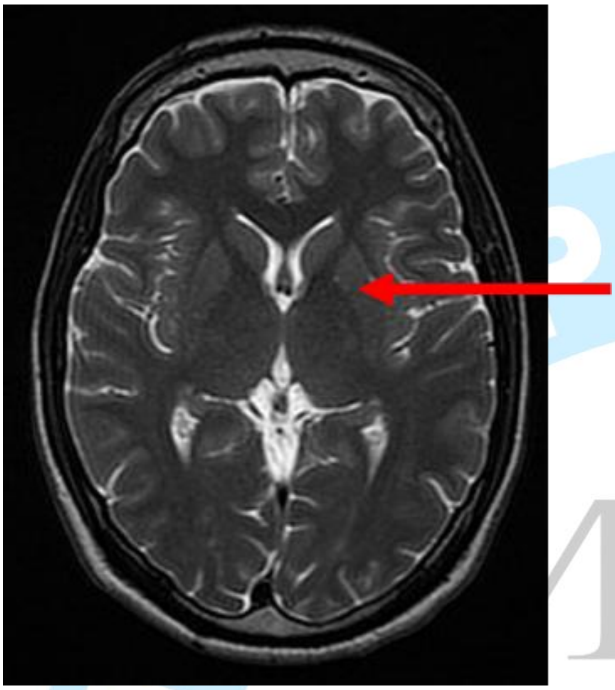
Q12. A swelling appears in the groin and descends into the scrotum through the deep inguinal ring. What type of hernia is this?
a) Direct inguinal hernia
b) Indirect inguinal hernia
c) Femoral hernia
d) Umbilical hernia

Q13. A patient has difficulty pushing against a wall. His scapula protrudes posteriorly. Which muscle is paralyzed?
a) Trapezius
b) Rhomboids
c) Serratus anterior
d) Latissimus dorsi

Q14. A patient has loss of direct and consensual light reflex in one eye when light is shone in it, but both reflexes are present when light is shone in the other eye. Which nerve is damaged?
a) Optic nerve
b) Oculomotor nerve
c) Trochlear nerve
d) Abducens nerve

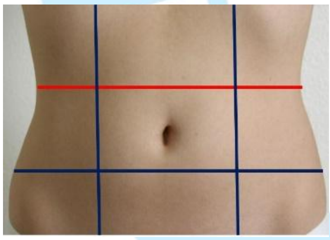
Q15. Identify the arrow marked structure :

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026



a) Thalamus
b) Lentiform nucleus
c) Caudate nucleus
d) Corpus callosum

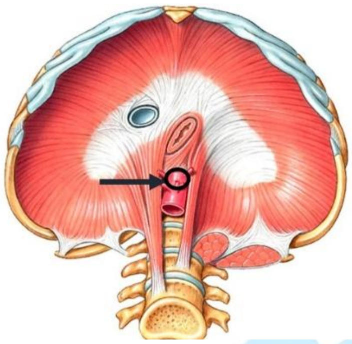
Q16. What is the vertebral level for the red coloured plane in the image ?



a) At junction of T12 & L1
b) Lower border of L1
c) Upper border of L2
d) Upper border of L3

Q17. The arrow marked opening in the diaphragm gives passage to which structure?

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026



a) Vagus nerve
b) Esophagus
c) Inferior vena cava
d) Thoracic duct

Q18. Regarding resting membrane potential physiology, all of the following statements are TRUE EXCEPT:

a) The resting membrane potential is determined predominantly by potassium permeability because resting membranes contain a greater number of potassium leak channels compared with sodium channels.
b) The sodium-potassium ATPase contributes indirectly to resting membrane potential maintenance by sustaining ionic gradients across the membrane through active transport.
c) Increasing extracellular potassium concentration generally reduces the magnitude of the resting membrane potential by decreasing the potassium diffusion gradient.
d) Resting membrane potential is generated primarily by rapid opening of voltage-gated sodium channels causing inward sodium current during the resting state.

Q19. Regarding action potential physiology, all of the following statements are TRUE EXCEPT:

a) Rapid depolarization during the upstroke of the action potential results primarily from opening of voltage-gated sodium channels causing inward sodium current.
b) The absolute refractory period occurs because voltage-gated sodium channels remain in an inactivated state despite ongoing membrane repolarization.
c) Saltatory conduction increases conduction velocity by allowing depolarization to jump between nodes of Ranvier in myelinated fibers.
d) Hyperpolarization during the refractory period results mainly from persistent inward sodium current through voltage-gated sodium channels.

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

Q20. Regarding neuromuscular junction physiology, all of the following statements are TRUE EXCEPT:

a) Acetylcholine released from presynaptic vesicles binds nicotinic receptors on the motor end plate, producing sodium influx and depolarization.
b) Acetylcholinesterase rapidly hydrolyzes acetylcholine within the synaptic cleft, terminating neurotransmission.
c) Calcium influx into the presynaptic terminal is required for exocytosis of acetylcholine-containing vesicles.
d) Nondepolarizing neuromuscular blockers produce paralysis primarily by causing persistent depolarization of the motor end plate.

Q21. Regarding sarcomere physiology, all of the following statements are TRUE EXCEPT:

a) Maximum active tension is generated at an optimal sarcomere length where actin and myosin overlap permits maximal cross-bridge formation.
b) The A band remains relatively constant in length during skeletal muscle contraction because I band slides into the A band.
c) ATP binding to myosin heads is required for detachment of actin-myosin cross bridges during the contraction cycle.
d) Muscle contraction occurs primarily because actin and myosin filaments themselves shorten substantially during force generation.

Q22. Regarding cardiac cycle physiology, all of the following statements are TRUE EXCEPT:

a) Isovolumetric contraction occurs after mitral valve closure but before aortic valve opening, during which ventricular pressure rises rapidly without volume change.
b) Ventricular filling occurs primarily during diastole and is facilitated by ventricular relaxation and pressure gradients between atria and ventricles.
c) Increased afterload generally reduces stroke volume because the ventricle must generate greater pressure before ejection occurs.
d) The second heart sound occurs due to opening of semilunar valves at the onset of ventricular systole.

Q23. Regarding Frank-Starling physiology, all of the following statements are TRUE EXCEPT:

a) Increased venous return stretches myocardial fibers, enhancing force generation and stroke volume within physiologic limits.

b) Frank-Starling behavior reflects improved actin-myosin interaction at optimal fiber length during increased preload.
c) Excessive ventricular dilation beyond physiologic range may eventually impair contractile efficiency and reduce cardiac output.
d) Frank-Starling effects depend primarily on sympathetic stimulation increasing heart rate rather than changes in ventricular filling.

Q24. A patient with progressive right-sided heart failure demonstrates a large venous pulsation synchronous with ventricular systole. Examination also reveals a pansystolic murmur increasing during inspiration. Regarding jugular venous pulse physiology, all of the following statements are TRUE EXCEPT:

a) The ‘a wave’ of JVP represents right atrial contraction occurring immediately before ventricular systole.
b) Cannon ‘a waves’ occur when atria contract against a closed tricuspid valve during AV dissociation.
c) Prominent ‘v waves’ may occur in tricuspid regurgitation because ventricular systole causes retrograde flow into the right atrium.
d) The ‘y descent’ reflects atrial contraction and normally corresponds to systolic emptying of the right atrium.

Q25. A septic patient with high fever and metabolic acidosis develops increased tissue oxygen extraction despite reduced arterial oxygen saturation. Regarding oxygen transport physiology, all of the following statements are TRUE EXCEPT:

a) Increased temperature shifts the oxyhemoglobin dissociation curve to the right, facilitating oxygen unloading.
b) Elevated 2,3-bisphosphoglycerate reduces hemoglobin affinity for oxygen and promotes tissue oxygen delivery.
c) Fetal hemoglobin has reduced affinity for oxygen compared with adult hemoglobin, facilitating placental oxygen unloading.
d) Acidosis shifts the oxyhemoglobin dissociation curve to the right through the Bohr effect.

Q26. A patient with pulmonary embolism develops acute dyspnea and hypoxemia despite relatively preserved lung mechanics. Arterial blood gas demonstrates respiratory alkalosis. Regarding ventilation-perfusion physiology, all of the following statements are TRUE EXCEPT:

a) Physiologic dead space increases when alveoli are ventilated but poorly perfused.
b) Ventilation-perfusion mismatch is a common cause of hypoxemia in pulmonary vascular disease.

c) Hypoxic pulmonary vasoconstriction attempts to divert blood flow away from poorly ventilated alveoli.
d) Areas with high ventilation-perfusion ratios characteristically produce severe hypoxemia that fully corrects with increased carbon dioxide retention.

Q27. Regarding glomerular filtration physiology, all of the following statements are TRUE EXCEPT:

a) Glomerular filtration rate depends partly on hydrostatic pressure gradients across the glomerular capillary membrane.
b) Constriction of the efferent arteriole may initially increase glomerular filtration pressure.
c) Plasma oncotic pressure opposes filtration across the glomerular membrane.
d) Filtration fraction decreases to zero whenever afferent arteriolar resistance falls because renal plasma flow rises proportionally more than filtration.

Q28. A patient with uncontrolled diabetes mellitus develops polyuria, polydipsia, and weight loss. Urine testing shows marked glycosuria. Serum glucose is far above the renal threshold. Regarding tubular reabsorption, all are TRUE EXCEPT:

a) Glucose is normally reabsorbed in the proximal convoluted tubule by sodium-glucose cotransporters.
b) Glycosuria occurs when filtered glucose load exceeds tubular transport maximum.
c) Proximal tubular glucose reabsorption is an active sodium-dependent process.
d) Glucose is normally reabsorbed mainly in the collecting duct under ADH control.

Q29. A patient with chronic lithium use develops polyuria and impaired urinary concentration. Regarding renal countercurrent physiology, all are TRUE EXCEPT:

a) The descending limb is highly permeable to water.
b) The thick ascending limb actively reabsorbs Na-K-2Cl but is impermeable to water.
c) The medullary gradient helps concentrate urine in the presence of ADH.
d) The ascending limb concentrates tubular fluid by reabsorbing water more than solute.

Q30. A dehydrated patient develops hypotension and reduced renal perfusion. Regarding RAAS physiology, all are TRUE EXCEPT:

a) Renin is released from juxtaglomerular cells in response to reduced renal perfusion.
b) Angiotensin II preferentially constricts efferent arterioles.

c) Aldosterone increases sodium reabsorption and potassium secretion.
d) ACE converts angiotensin II into angiotensin I in pulmonary circulation.

Q31. A patient with SIADH has hyponatremia, low serum osmolality, and concentrated urine. Regarding ADH, all are TRUE EXCEPT:

a) ADH increases aquaporin-2 insertion in collecting ducts.
b) ADH secretion is stimulated by increased plasma osmolality.
c) ADH promotes free water retention.
d) ADH produces dilute urine by reducing collecting duct water permeability.

Q32. A patient with renal tubular acidosis has persistent metabolic acidosis and impaired urinary acidification. All are TRUE EXCEPT:

a) Distal nephron secretes hydrogen ions to acidify urine
b) Ammonium trapping (also k/a non-ionic diffusion of ammonia) helps excrete acid.
c) New bicarbonate generation is linked to renal acid excretion in the collecting duct
d) Renal compensation for metabolic acidosis occurs mainly by reducing hydrogen ion secretion

Q33. A patient with severe hyperkalemia develops ECG changes. All are TRUE EXCEPT:

a) Hyperkalemia makes resting membrane potential less negative.
b) Severe hyperkalemia can cause arrhythmias.
c) Insulin shifts potassium into cells.
d) Aldosterone decreases potassium secretion in the cortical collecting duct.

Q34. A patient after thyroid surgery develops perioral numbness and carpopedal spasm. Regarding calcium physiology, all are TRUE EXCEPT:

a) PTH increases serum calcium.
b) PTH increases renal phosphate reabsorption.
c) Vitamin D increases intestinal calcium absorption.
d) Calcitonin is the main hormone required for minute-to-minute calcium maintenance.

Q35. In Menke's kinky hair syndrome there is defective collagen cross-linking that is primarily due to dietary deficiency of:

a) Zinc
b) Copper
c) Calcium

d) Magnesium

Q36. The following POCT for blood sugar estimation is based on:

a) Glucose is converted to glucuronic acid
b) Glucose is converted to gluconic acid
c) Glucose oxidase is specific for beta anomer of glucose
d) This is type of reductometric metric method

Q37. Which of the following hormone inhibits gluconeogenesis ?
a) Growth hormone
b) Corticosteroids
c) Insulin
d) Glucagon

Q38. The infant is suffering from one of the following abnormalities in the picture depicted below:

a) Oil drop Cataract
b) Snow Flake Cataract

c) Sunflower cataract
d) Kayser Fleischer ring

Q39. Toxicity of cholera toxin is due to:
a) Decrease in activity of Gs protein
b) Decrease in activity of Gi protein
c) ADP ribosylation of Gs alpha subunit
d) ADP ribosylation of Gi alpha subunit

Q40. Ramesh a 36 yr old school teacher presents to the OPD with a history of lower back pain and painful urination. On work up it reveals normocytic normochromic anemia with hypercalcemia. The physician asks for a special urine test in which Bradshaw's test comes positive but Jacobson & Milner test comes negative. To evaluate further serum protein electrophoresis was done that shows the following:

What is the most likely diagnosis?
a) Monoclonal gammopathy of undetermined significance
b) Multiple Myeloma
c) Guillain–Barré syndrome
d) AIDS

Q41. During the management of Classical Phenylketonuria the mainstay of first line therapy is :
a) Replacement of the defective enzyme
b) Replacement of the deficient product
c) Limiting the substrate for deficient enzyme
d) Giving the missing amino acid through dietary supplementation

Q42. The following are salient features of allosteric enzymes except:
a) They are usually multiple subunit enzymes
b) They are involved in metabolic regulation
c) They do not show any change in Km and Vmax in the presence of modifiers
d) On binding of the allosteric inhibitor, usually Km is increased and Vmax is decreased

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

Q43. A 40 years old middle aged person presented to the OPD with complains of lethargy, weakness and shortness of breath while climbing stairs or walking a mile. His routine investigations reveal high homocysteine level. Urine for methyl malonyl CoA was normal though. What is the most likely vitamin deficiency that would have led to this condition?

a) Niacin
b) Pyridoxine
c) Folic Acid
d) Cobalamin

Q44. A 4 months old child presents to the Pediatric OPD with following type of rash around genitalia.

Which of the following mineral deficiency is most likely to be associated with this?

a) Cobalt deficiency
b) Iron deficiency
c) Copper deficiency
d) Zinc deficiency

Q45. A child during a workup at well baby clinic is identified to suffer from frequent fasting hypoglycemia. Further evaluation reveals an inborn error in carbohydrate metabolism where the underlying causes is inability to perform both glycogenolysis & gluconeogenesis. Which of the following enzyme is missing in this child?

a) Glycogen phosphorylase
b) Glucokinase
c) Glucose 6 Phosphatase
d) Glycogen synthase

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

Q46. Products of glucuronic acid pathway in human beings are all except :
a) Vitamin C
b) Glucuronic acid
c) Pentoses
d) NADH

Q47. Lactic acidosis can be seen due to all of the following enzyme dysfunction except?
a) Phosphoenolpyruvate Carboxykinase
b) Pyruvate Dehydrogenase complex
c) Pyruvate Carboxylase
d) Pyruvate kinase

Q48. All of the following statements about Lipoprotein Lipase are true except :
a) Found in adipose tissue
b) Not found in liver
c) Deficiency leads to hypertriglyceridemia
d) Doesn't require CII as its cofactor

Q49. False regarding gout is:
a) MSU crystals are rhombic shaped crystals with positive birefringence
b) Due to increased metabolism of purines
c) Uric acid levels may not be elevated
d) Has a predilection for the great toe

Q50. One of the following is regulated by repression-derepression:
a) ALA synthase by heme
b) Succinate dehydrogenase by malonate
c) Cytochromes by cyanide
d) Glycogen phosphorylase by glucagon

Q51. The anti metabolite, methotrexate acts by:
a) Non competitive inhibition
b) Formation of a covalent bond between enzyme and inhibitor
c) Increasing the Vmax of the reaction
d) Competitive inhibition

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

Q52. A 38-year-old man presents with a 3-month history of low-grade fever, night sweats, weight loss, and chronic cough with occasional blood-streaked sputum. He recently returned from living in a crowded shelter. Chest X-ray reveals cavitary lesions in the upper lobe of the right lung. A biopsy obtained from the lesion shows granulomatous inflammation with multinucleated giant cells and a central area of amorphous, granular, eosinophilic debris lacking preserved cellular architecture.

Which type of necrosis is most likely responsible for the lesion seen in this patient?

a) Coagulative necrosis
b) Liquefactive necrosis
c) Caseous necrosis
d) Fat necrosis

Q53. A 28-year-old woman is diagnosed with lobar pneumonia. As the inflammatory response intensifies, cytokines like TNF and IL-1 act on the local pulmonary capillary endothelium. This causes the transient and weak binding of leukocytes to the vessel wall, resulting in a "tumbling" or "rolling" motion along the surface. Deficiency in which of the following molecules would most likely impair this specific initial step of leukocyte recruitment?

a) Selectins
b) Integrins
c) C5a and C3a
d) NADPH Oxidase

Q54. A 42-year-old woman with a history of recurrent viral infections undergoes an immunological workup. Laboratory studies reveal decreased activity of natural killer (NK) cells, which are responsible for destroying virus-infected and malignant cells without prior sensitization. Flow cytometry is performed to identify the affected lymphocyte population. The abnormal cells are found to lack surface CD3 expression but strongly express characteristic NK-cell markers. Which of the following CD marker combinations is most commonly associated with natural killer (NK) cells?

a) CD3 and CD4
b) CD19 and CD20
c) CD16 and CD56
d) CD40 and CD154

Q55. A 50-year-old patient undergoes a colonoscopy, which reveals a large sessile polyp in the sigmoid colon. Molecular profiling of the tissue reveals a loss-of-function mutation in a key tumor suppressor protein. In its active, hypo phosphorylated state, this protein binds to the E2F transcription factor, effectively "braking" the cell cycle and preventing the cell from transitioning from the G1 phase to the S phase. Which of the following proteins is responsible for this regulatory "checkpoint"?

a) p53
b) RB protein
c) MYC
d) BCL-2

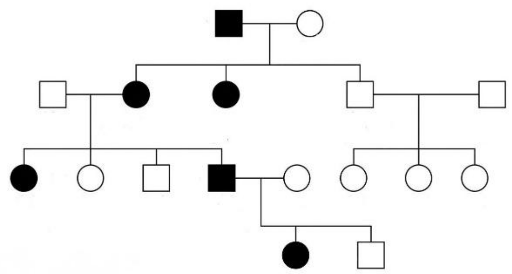
Q56. A 64-year-old man with a 40-pack-year smoking history presents with persistent cough, weight loss, and progressive weakness. Over the past few weeks, he has also developed confusion, nausea, and muscle cramps. Laboratory investigations reveal severe hyponatremia with low plasma osmolality and inappropriately concentrated urine. Chest imaging demonstrates a centrally located hilar mass, and bronchoscopic biopsy confirms a malignant tumor composed of small hyperchromatic cells with scant cytoplasm and neuroendocrine features.

The patient's electrolyte abnormality is suspected to be due to ectopic hormone production by the tumor.

Which of the following paraneoplastic syndromes is most commonly associated with this malignancy?

a) Hypercalcemia due to parathyroid hormone–related peptide (PTHrP) secretion
b) Cushing syndrome due to ectopic ACTH production
c) Syndrome of inappropriate ADH secretion (SIADH) due to ectopic ADH production
d) Polycythemia due to excess erythropoietin production

Q57. Comment on the pedigree analysis :

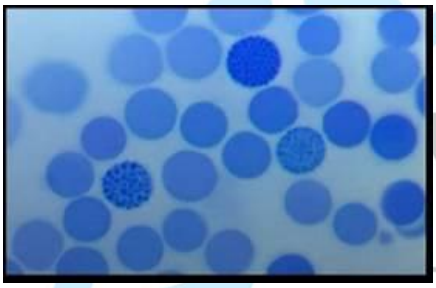


a) AR
b) AD
c) XLR
d) XLD

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

Q58. A 14-year-old boy of Southeast Asian descent is brought to the hematology clinic with fatigue, mild jaundice, and recurrent episodes of weakness. Physical examination reveals pallor, splenomegaly, and mild hepatomegaly. Laboratory investigations show microcytic hypochromic anemia with increased reticulocyte count. Peripheral blood smear demonstrates target cells and numerous Heinz body–containing erythrocytes on supravital staining. Hemoglobin electrophoresis reveals the presence of β4 tetramers.

Which of the following best describes the type of alpha-thalassemia and the number of α-globin gene deletions present in this patient?



a) Silent carrier state — deletion of 1 α-globin gene
b) Alpha-thalassemia trait — deletion of 2 α-globin genes
c) Hemoglobin H disease — deletion of 3 α-globin genes
d) Hydrops fetalis with Hb Bart’s — deletion of 4 α-globin genes

Q59. 48-year-old man presents with progressive fatigue, fever, recurrent infections, and easy bruising for the past several weeks. Physical examination reveals pallor, petechiae, and mild hepatosplenomegaly. Laboratory investigations show pancytopenia with a markedly elevated blast count in the peripheral blood. Bone marrow aspiration demonstrates numerous myeloblasts containing slender, reddish needle-shaped cytoplasmic inclusions that stain positive for myeloperoxidase. Which of the following is the hallmark microscopic finding described in this patient?
a) Smudge cells
b) Reed–Sternberg cells
c) Auer rods
d) Howell–Jolly bodies

Q60. A 67-year-old man presents with persistent lower back pain, fatigue, and recurrent respiratory infections for the past several months. Laboratory investigations show normocytic anemia, elevated serum creatinine, and hypercalcemia. Serum protein electrophoresis demonstrates a monoclonal spike, and urine analysis is positive for Bence Jones proteins. Skeletal survey reveals multiple punched-out lytic lesions in the skull and vertebrae. Bone marrow biopsy shows clonal proliferation of plasma cells.

The pathogenesis of Multiple myeloma involves activation of signaling pathways promoting plasma cell proliferation, including mutations affecting the MAP kinase pathway, commonly involving a specific light-chain–associated oncogene.

Which of the following genetic abnormalities is most commonly associated with activation of the MAP kinase pathway in multiple myeloma?

a) MYC translocation involving chromosome 8
b) KRAS mutation associated with MAP kinase activation
c) BCR-ABL fusion gene formation
d) JAK2 V617F mutation

Q61. A 14-year-old girl is brought to the hematology clinic with a history of frequent nosebleeds, easy bruising, and excessive bleeding after minor cuts since childhood. She attained menarche 1 year ago and has experienced unusually heavy menstrual bleeding. There is a family history of similar bleeding tendencies in her mother.

On examination, multiple ecchymotic patches are present over the extremities. Laboratory investigations reveal:

Normal platelet count

Prolonged bleeding time

Mildly prolonged activated partial thromboplastin time (aPTT)

Platelet aggregation studies show defective platelet adhesion.

Which of the following best explains the underlying defect in this patient?

a) Deficiency of factor VIII causing impaired coagulation cascade only
b) Defective platelet adhesion due to deficiency of von Willebrand factor
c) Autoimmune destruction of circulating platelets
d) Deficiency of glycoprotein IIb/IIIa causing impaired platelet aggregation

Q62. A 7-year-old boy is brought to the pediatric neurology clinic with progressive headache, morning vomiting, and difficulty maintaining balance for the past 2 months. His parents also report frequent falls and unsteady gait. On neurological examination, the child has truncal ataxia and nystagmus. MRI of the brain reveals a midline cerebellar mass compressing the fourth ventricle, causing obstructive hydrocephalus. Histopathological examination of the tumor shows densely packed small round blue cells forming Homer-Wright rosettes.

Which of the following mutations is most commonly associated with the development of medulloblastoma?

a) N-myc mutation
b) RET proto-oncogene mutation
c) JAK2 mutation
d) BCR-ABL translocation

Q63. A 52-year-old female presents with frequent headaches, dizziness, and a distressing "burning" sensation in her hands and feet associated with reddish-blue discoloration (erythromelalgia). She has no history of smoking, chronic infections, or recent surgery. On examination, her spleen is palpable 2 cm below the left costal margin. Platelet Count: 9,80,000/µL (Markedly elevated). Bone Marrow Biopsy: Shows hypercellularity with clusters of enlarged, "staghorn" megakaryocytes. Molecular Testing: Positive for the JAK2 V617F mutation. After ruling out reactive causes of high platelets (like iron deficiency or inflammation), what is the most likely diagnosis?

a) Polycythemia Vera
b) Essential Thrombocythemia (ET)
c) Chronic Myeloid Leukemia (CML)
d) Primary Myelofibrosis (PMF)

Q64. A 56-year-old chronic smoker presents with a long history of productive cough lasting more than 3 months each year for the past 2 years. Physical examination reveals coarse crackles, and chest X-ray shows increased bronchovascular markings. Bronchial biopsy demonstrates marked mucous gland hypertrophy. Which measurement is used to assess the severity of this gland enlargement?

a) Kerley index used to assess interstitial edema in heart failure
b) Reid index measuring mucous gland thickness relative to bronchial wall thickness
c) Karnofsky index evaluating functional status in chronic disease
d) Apgar score assessing neonatal health immediately after birth

Q65. A 23-year-old male is brought with agitation, dry mouth, tachycardia, urinary retention, and blurred vision after ingesting unknown plant seeds. On examination, his skin is hot, flushed, and dry with dilated pupils. Bowel sounds are reduced. The clinical picture is suggestive of an anticholinergic toxidrome. Which of the following is the most likely cause?

a) Opioid overdose
b) Physostigmine toxicity

c) Datura ingestion
d) Organophosphate poisoning

Q66. A 60-year-old male with known heart failure presents with worsening dyspnea and easy fatigability. Echocardiography shows reduced ejection fraction with evidence of ventricular remodeling. A drug is started that improves survival by reducing sympathetic activity and limiting cardiac remodeling. Which of the following is the most appropriate choice?
a) Dobutamine
b) Furosemide
c) Milrinone
d) Carvedilol

Q67. A hypertensive patient is started on an ACE inhibitor. After a few weeks, he complains of persistent dry cough and mild hyperkalemia. The physician reassures him about common adverse effects. Which of the following is NOT typically associated with ACE inhibitor therapy?
a) Angioedema
b) Hypokalemia
c) Cough
d) Hyperkalemia

Q68. A newly developed antiarrhythmic drug is found to prolong action potential duration and increase refractory period in cardiac myocytes. ECG shows QT prolongation without affecting depolarization velocity. This pharmacological profile corresponds to which class of antiarrhythmics?
a) Class I
b) Class II
c) Class IV
d) Class III

Q69. A patient undergoing chemotherapy develops hyperuricemia, hyperkalemia, and acute kidney injury within days of treatment initiation. Tumor lysis syndrome is suspected. A drug is administered that rapidly converts uric acid into a soluble metabolite. Which of the following is most appropriate?
a) Febuxostat
b) Alendronate
c) Rasburicase
d) Colchicine

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

Q70. A patient presents with fever, cough, diarrhea, and confusion after cleaning an air-conditioning unit. Chest imaging shows patchy infiltrates. Legionella infection is suspected. Which of the following drugs is the treatment of choice in this condition?
a) Norfloxacin
b) Daptomycin
c) Ceftobiprole
d) Azithromycin

Q71. A patient with bronchial asthma presents with acute exacerbation and is started on bronchodilators. One of the prescribed drugs has a slow onset and is not useful for immediate relief. Which of the following is least suitable for acute symptom control?
a) Salbutamol
b) Terbutaline
c) Formoterol
d) Salmeterol

Q72. A 25-year-old male presents with sudden palpitations and dizziness. ECG shows narrow complex tachycardia suggestive of supraventricular tachycardia. A rapid IV drug is administered causing transient AV nodal block and immediate termination of arrhythmia. Which drug was used?
a) Verapamil
b) Amiodarone
c) Adenosine
d) Lidocaine

Q73. A patient develops excessive bleeding after thrombolysis with streptokinase for myocardial infarction. An antifibrinolytic agent is administered to reverse this effect. Which of the following drugs is most appropriate?
a) Vitamin K
b) Rutin
c) Noradrenaline
d) Tranexamic acid

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

Q74. A patient presents with persistent nausea and vomiting and is started on a drug that acts as a dopamine antagonist and 5-HT4 agonist, enhancing gastric motility. Which of the following drugs is most likely prescribed?

a) Ondansetron
b) Ibutilide
c) Metoclopramide
d) Benzhexol

Q75. A patient on long-term inhaled corticosteroids for asthma develops oral candidiasis. To reduce systemic exposure, the physician switches to a prodrug that is activated within the lungs. Which of the following is the most appropriate choice?

a) Fluticasone
b) Budesonide
c) Mometasone
d) Ciclesonide

Q76. A cancer patient undergoing chemotherapy develops severe nausea and vomiting. A drug is administered that blocks serotonin receptors in the chemoreceptor trigger zone. Which of the following drugs is most appropriate?

a) Doxylamine
b) Domperidone
c) Scopolamine
d) Palonosetron

Q77. A patient treated for nausea develops acute dystonia, restlessness, and muscle rigidity. These extrapyramidal symptoms are due to dopamine receptor blockade in the CNS. Which drug is most likely responsible?

a) Ondansetron
b) Doxylamine
c) Metoclopramide
d) Domperidone

Q78. A 4-year-old child exposed to a viral infection in kindergarten presents with fever, maculopapular rash and Forchheimer spots. The rash would be:

a) Centripetal

b) Centrifugal
c) Dermatomal
d) Cephalocaudal

Q79. A 54-year-old chronic alcoholic is brought with high fever, dyspnoea at rest and chest pain. Gram-negative non-motile bacilli are seen. All are correct except:
a) Etiology - Klebsiella pneumoniae
b) Red currant jelly sputum
c) Bishop scoring to be done
d) Chest X-ray will reveal lobar consolidation

Q80. A 21-year-old lactating mother develops fever, redness and a lump in the right breast. She has a resistant infection by Gram-positive cocci. Resistance to cell-wall antibiotics is suspected, and later treatment requires an antibiotic acting on the 50S bacterial ribosome. Which genes are involved in resistance?
a) rpoB gene, gag gene
b) mecA gene, van gene
c) inhA gene, katG gene
d) BCR-ABL gene, c-myc gene

Q81. An 11-year-old girl on 3-weekly benzathine penicillin prophylaxis presents with dyspnoea, fatigue, bilateral pedal edema, bilateral basal crepitations and an Austin Flint murmur. What is the most appropriate diagnosis?
a) Hypertrophic cardiomyopathy + pulmonary artery stenosis
b) Congestive cardiac failure + aortic regurgitation
c) Torsades de pointes + complete heart block
d) Cardiac tamponade + pulmonary thromboembolism

Q82. A 25-year-old COVID-positive G2P1L1 undergoes LSCS. Post-surgery, equipment is sterilized at 121°C for 20 minutes at 15 psi, and glassware is sterilized at 160°C for 2 hours. Identify the methods:
a) Plasma sterilization, inspissation
b) Autoclave, hot air oven
c) Incineration, tyndallization
d) UV irradiation, gamma ray sterilization

Q83. A 41-year-old elderly primigravida is given pregabalin to manage a common complication of infection by a virus reactivated from the ophthalmic branch of the trigeminal ganglia. What are the diagnosis and etiology?

a) Post-traumatic epilepsy, neurocysticercosis
b) Post-herpetic neuralgia, varicella zoster virus
c) Post-partum depression, herpes simplex virus
d) Post-operative pain, surgical manipulation

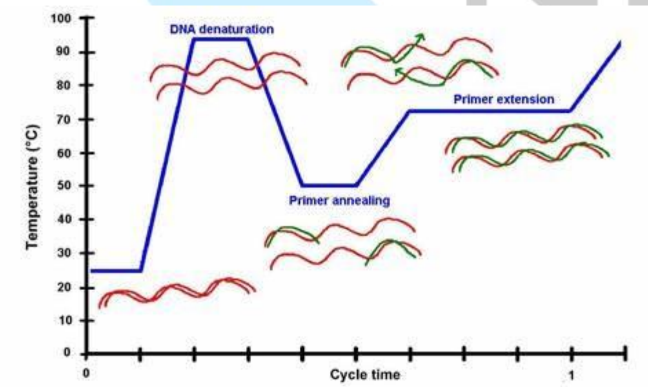
Q84. A 77-year-old patient with fever, headache and altered mental status is diagnosed with brain abscess. Gram stain reveals purple bacteria with thick peptidoglycan and teichoic acid. All are possible except:

a) Pneumococcus
b) Meningococcus
c) Enterococcus
d) Staphylococcus

Q85. A 12-year-old child presents with fever, throat pain, compromised mouth opening and change in voice. The ENT surgeon diagnoses quinsy. Identify the correct combination:

a) Streptococcus pneumoniae, parapharyngeal abscess, ciprofloxacin
b) Streptococcus pyogenes, peritonsillar abscess, penicillin
c) Streptococcus agalactiae, submandibular abscess, erythromycin
d) Streptococcus viridans, otitis, framycetin

Q86. A 44-year-old unmarried virgin female undergoes screening through the procedure shown below in the gynecology OPD. Identify the procedure:



a) Pap smear

b) PCR
c) VIA
d) VILI

Q87. A couple on a date in Buddha Garden, Delhi, is bitten by the creature shown below. All statements are correct except:

a) Transmits dengue, chikungunya, yellow fever, zika
b) Larva is flat and parallel to surface of water
c) Flight range 100 metres
d) Lives in urban areas breeding in fresh rain water

Q88. A 63-year-old PLWHA presents with fever, dyspnoea, dry cough and chest tightness. CT chest shows ground-glass opacities. Grocott stain of sputum reveals a ping-pong ball appearance. What are the diagnosis and treatment?
a) Aflatoxicosis, antitoxin
b) Zygomycosis, voriconazole
c) Pneumocystis jirovecii pneumonia, cotrimoxazole
d) Aspergilloma, amphotericin B

Q89. A 33-year-old man had trauma to one eye in a road accident. After 3 months, he develops loss of vision, pain, photophobia and redness in the other eye. What are the diagnosis and reason?
a) Chorioretinitis, occipital lobe damage
b) Sympathetic ophthalmia, sequestered antigen
c) Iridocyclitis, optic nerve damage
d) Strabismus, oculomotor nerve damage

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

Q90. A 32-year-old develops a dead-end accidental zoonotic infection related to dogs. CT abdomen reveals a cystic lesion in the right liver lobe. He is treated by PAIR therapy. What are the diagnosis and drug of choice?

a) Amoebic liver abscess, metronidazole
b) E. coli liver abscess, ceftriaxone
c) Hydatid cyst, albendazole
d) Hepatitis C, sofosbuvir

Q91. A man Mr 'A" has seen a person with knife looking for Mr X. later on it is learnt that Mr X gets stabbed. Mr 'A' tells the same in the court. What kind of evidence is he giving?
a) Direct oral evidence
b) Documentary evidence
c) Hearsay evidence
d) Indirect oral evidence

Q92. If below is the description of a death certificate. What is the antecedent cause of death?

	Cause of death	Interval
I	Respiratory failure Chronic COPD Smoking history 30 years	2 hours 5 hours 10 years
II	Hypertension	22 years

a) Respiratory failure
b) Chronic COPD
c) Hypertension
d) Smoking

Q93. A man gets sexual gratification by stealing and playing with the clothes of his neighbor female, which she puts out to dry after washing. What is this called as?

a) Exhibitionism
b) Fetichism
c) Algolagnia
d) Froturrism

Q94. A person has suffered trauma due to which his visual acuity becomes from 6/5 to 6/18. True statement is?

a) This is simple hurt as per Sec 114 BNS
b) This is grievous hurt as per Sec 116 BNS
c) This is dangerous injury
d) Decrease of vision is not a hurt

Q95. A 34 years female, mother of two, has uterine pathology. Her doctor suggests hysterectomy. She sits with her surgeon who explains her everything about the surgery and after understanding everything she consents for the procedure. This is example of?

a) Blanket consent
b) Informed consent
c) Expressed consent
d) Implied consent

Q96. The minimum age to give consent in India is?

a) 12 years
b) 10 years
c) 7 years
d) 18 years

Q97. Sexual perversion in which sexual gratification is obtained or increased by the suffering of pain:

a) Eonism
b) Fetishism
c) Sadism
d) Masochism

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

Q98. All are features of male skull except?
a) Prominent frontal eminences
b) Heavy zygoma
c) Prominent supra-orbital ridges
d) triangular thick mastoids

Q99. Not a feature of an antemortem burn?
a) Lines of redness
b) Protein rich blisters
c) Pugilistic attitude
d) Vital reaction

Q100. In a case of death due to strangulation, the preferred body opening incision would be?
a) I shaped incision
b) Y shaped incision
c) Modified Y shaped incision
d) X shaped incision

Q101. A 6-week-old infant receives the pentavalent vaccine. After 6 hours, the child develops fever, irritability, and localized swelling at the injection site. The symptoms resolve spontaneously within 24 hours without intervention. Which of the following best explains this reaction?

a) Vaccine failure
b) Severe adverse event
c) Expected minor side effect
d) Anaphylaxis

Q102. A primary health centre routinely reports cases of malaria, dengue, and tuberculosis to the district health office based on patients who present to the facility. No active search for cases is conducted in the community. The data collected depends largely on healthcare-seeking behaviour and routine reporting by clinicians. Which type of surveillance is being described?

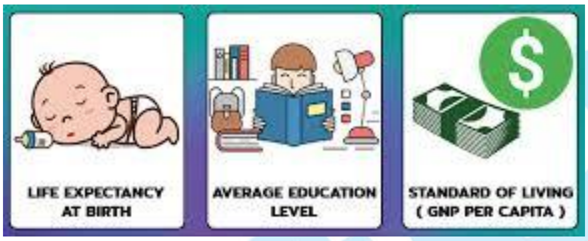
a) Active surveillance
b) Passive surveillance
c) Sentinel surveillance
d) Syndromic surveillance

Q103. In a community, only patients with severe symptoms of tuberculosis seek medical care and are recorded in official statistics. However, many individuals with latent or mild disease remain undetected. Which of the following represents the “tip of the iceberg” in this scenario?

a) Latent infections
b) Subclinical cases
c) Diagnosed symptomatic cases
d) Carriers

Q104. A state reports zero indigenous cases of a particular infectious disease for the past 3 years due to sustained vaccination and surveillance activities. However, occasional imported cases continue to occur due to travel from endemic regions. The health authorities continue active surveillance and immunization programs to prevent re-establishment of transmission. Which term best describes this situation?
a) Eradication
b) Elimination
c) Control
d) Extinction

Q105. A policymaker compares development across countries using an index that incorporates life expectancy at birth, education (mean and expected years of schooling), and gross national income per capita. This index is widely used by international organizations to rank countries. Which index is being described?



a) PQLI
b) DALY
c) HDI
d) QALY

Q106. In a rural village, residents begin to develop cases of diarrhoea over several weeks. Investigation reveals contamination of a water supply that continues for an extended period. New cases keep appearing as long as the population is exposed to the contaminated source.

Which type of epidemic is most likely?

a) Point source epidemic
b) Continuous common source epidemic
c) Propagated epidemic
d) Cyclical epidemic

Q107. Which of the following is the best indicator of efficiency of contraceptives?

→

a) Pearls index
b) Sullivan index
c) Life table analysis
d) Chandlar index

Q108. Which of the following criteria is sufficient to start MDT?

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

1. More than five hypopigmented patches
2. Hypopigmented patch with sensory loss
3. Peripheral nerve thickening
4. Peripheral nerve thickening with sensory loss
5. Skin biopsy demonstrating bacilli

Which of following is/are correct?

a) 1, 2, 3 and 5
b) 2, 4 and 5
c) 2 and 5
d) All are correct

Q109. Health centre reports high vaccine wastage rates along with low full immunization coverage. On further analysis, it is found that vaccines are frequently opened but not fully utilized due to poor session planning and irregular attendance of beneficiaries. Which of the following interventions would best address this issue?

a) Increase vaccine supply
b) Improve session planning and beneficiary mobilization
c) Reduce number of vaccines given
d) Stop outreach services

Q110. A 20-year-old patient is presented with single Hypopigmented and hypo anaesthetic lesion along with thickened single Nerve with lots of sensation, what will be your advice, as per recent WHO recommendations?

Pack A, Red Colour - Pack B, yellow colour

Options are:

a) Pack A for 12 months

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

b) Pack B for 6 months
c) Pack A for 6 months
d) Pack B for 12 months

Q111. Which of the following statement is FALSE regarding Rabies immunoglobulin?

a) Most of immunoglobulin is given in deltoid
b) Given at dose of 20 IU/kg for human immunoglobulins
c) Not given beyond 7 days of rabies vaccination
d) If human IG not available give Equine IG

Q112. Method of data collection in epidemiological study is done in:
a) Analytical statistics
b) Descriptive statistics
c) Theoretical statistics
d) Inferential statistics

Q113. A national health campaign uses specific logos and symbols to improve awareness, recognition, and participation among the population. These visual clues help people easily identify national health related programs. Which concept is best illustrated?

a) Surveillance
b) Health education through IEC
c) Demonstration
d) Propoganda

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

Q114. A public health poster shows given images to spread awareness about related diseases in India and different national programs. Which is correct about given image?

a) National TB elimination program
b) National leprosy elimination program
c) National leprosy eradication program
d) National AIDS control program

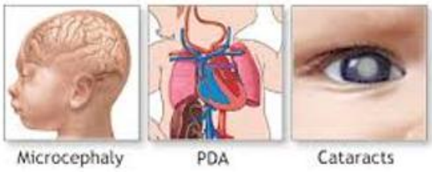
Q115. A child participates in a nationwide immunization campaign where two drops of vaccine is administered twice in a year at interval of 4 weeks. This campaign symbolizing protection against a crippling disease which is not present in India at present. Incorrect about this disease :

a) This program prefers use of OPV over IPV
b) Age group- 0-15 years
c) Described disease is in phase of elimination in India
d) Reverse cold chain is applied for transfer of stool sample of suspected case

Q116. A 10month HIV positive child presents with cough, fast breathing, and chest indrawing. As per WHO classification correct statement is :
a) Green category of IMNCI- pneumonia
b) Yellow category of IMNCI- pneumonia
c) Red category of IMNCI- severe pneumonia
d) Pink category of IMNCI- severe pneumonia

Q117. A pregnant woman develops mild fever and rash. Later, the baby is born with congenital heart defects and cataracts. All are correct about this disease except :

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

 → 

a) Most dangerous period of infection leading to malformation is first trimester of pregnancy
b) This complication is absolutely preventable by giving single dose of vaccine in first trimester to all pregnant women
c) This complication is absolutely preventable by checking for Ig G antibody before conception in each reproductive woman
d) Vaccine given for prevention is live attenuated with strain – RA27/3

Q118. A child presents with fever, neck stiffness, and altered consciousness to medical college. keeping in mind the diagnosis of meningitis lumber puncture test done and CSF analysis shows meningococcal meningitis infection. Incorrect about this disease :
a) Shows iceberg phenomenon
b) Disease is having very low case fatality rate as 10% only even in untreated patients
c) Prevention strategy- vaccination- chemoprophylaxis- early diagnosis and treatment
d) Drug of choice for chemoprophylaxis in pregnant lady is ceftriaxone

Q119. 5month infant presents with sudden onset fever, myalgia, headache, and cough during winter season and diagnosed as having influenza, correct statement about its treatment :
a) Oseltamivir 75 mg twice /day for 5 days
b) Oseltamivir 20 mg twice /day for 5 days
c) Oseltamivir 75 mg once /day for 10 days
d) Oseltamivir 20 mg once /day for 10 days

Q120. A child presents with fever, cough, conjunctivitis, and maculopapular rash starting from face and spreading downward with given spots on buccal mucosa. All are correct about this disease except :

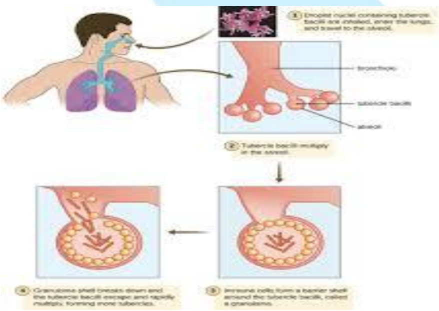
a) Goal of GOI- elimination by 2026
b) WHO elimination strategy- catch up- keep up- follow up
c) Period of communicability is 2 days before rash to 5 days after rash
d) Vaccine preventable disease and vaccine used is live attenuated vaccine

Q121. 1year child weight 10kg presents with acute watery diarrhoea. There is mild dehydration. The mother is advised oral rehydration therapy. Correct statement is
a) 750 ml ORS of 311meq/l osmolarity within 4 hours
b) 1000 ml ORS of 311meq/l osmolarity within 4 hours
c) 750 ml ORS of 245meq/l osmolarity within 4 hours
d) 1000ml ORS of 245meq/l osmolarity within 4 hours

Q122. A farmer sustains a deep contaminated wound while working in the field. He has no history of tetanus immunization. What is the most appropriate management immediately

a) TT vaccine only
b) Tetanus immunoglobulin only
c) TT + TIG
d) Td + TIG

Q123. A patient diagnosed with pulmonary tuberculosis is started on directly observed treatment (DOTS) to ensure adherence and cure. All are correct about this program except :



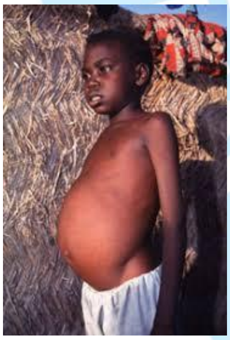
a) Compulsory CBNAAT in every patient before start of treatment
b) Treatment regimen for rifampicin sensitive cases- 4HRZE, 2HRE

c) Nikshay portal for TB surveillance
d) 1000Rs/month under Nikshay Poshan Yojna to all patients irrespective of type of TB till treatment completion

Q124. A patient is diagnosed with HIV infection who had received multiple blood transfusion earlier and is started on antiretroviral therapy under a national program. The program also includes prevention, counselling, and testing services. All are correct about this program except :

a) Counselling, and testing services are available at DMC centres
b) 1st line ART drugs- tenofovir, lamivudine, dolutegravir started immediately after diagnosis irrespective of CD4 count
c) 3month TB preventive therapy given as weekly isoniazid and rifapentine
d) On suspicion of TB in HIV individual investigation of choice is- CBNAAT

Q125. A patient from Bihar presents with prolonged fever, hepatosplenomegaly, and weight loss. Diagnosis confirms visceral leishmaniasis. The national program aims to eliminate disease by 2027. All are incorrect about this program except :



a) Elimination criteria- prevalence- < 1/10,000 population
b) Vector responsible- Phlebotomus fly which can only fly up to 6 inches above ground level
c) ASHA conduct quarterly fort night search for detection of hidden cases in community
d) Drug of choice- lyposomal amphotericin B 10mg/kg body weight as single oral dose

Q126. A group of 5,000 normotensive individuals were classified based on their salt intake (high vs low) in two equal halves and followed for 5years to observe the development of hypertension. At end it was observed that individuals with high salt intake developed hypertension 4 times more than individuals with low salt intake. Correct about this study?

a) Case-control study- ODD ratio-4
b) Cohort study- relative risk-4
c) Cohort study- attributable risk- 40%
d) Cohort study-population attributable risk- 40%

Q127. A 28-year-old woman opts for a non-hormonal, once-weekly oral contraceptive method provided under the national family welfare program. The drug acts as a selective estrogen receptor modulator and prevents implantation without affecting ovulation significantly.Which contraceptive is being described?

a) MALA-N
b) CHAYA
c) ANTARA
d) MIRENA

Q128. A 22-year-old woman presents to the emergency department 24 hours after unprotected sexual intercourse. She is not using any regular contraceptive method and is anxious to prevent pregnancy. She has no contraindications to hormonal therapy. The doctor prescribes a single tablet as oral medication that is most effective when taken within 72 hours. Which is the above drug ?

a) Levonorgestrel- 1.5mcg tablet
b) Levonorgestrel- 1.5mg tablet
c) Levonorgestrel- 0.75mcg tablet
d) Levonorgestrel- 0.75mg tablet

Q129. A patient with suspected appendicitis is referred from a primary health centre to a community health centre where surgical facilities and specialist doctors are available. Incorrect about this cent :

a) First referral unit
b) Population coverage- 1,00,000
c) Availability of full-time ophthalmologist
d) Availability of blood storage facility

Q130. Incorrect match for program and beneficiary:

a) ICDS- 0-6year children
b) ICDS- school going adolescent girls
c) RKSK- 10-19years adolescent
d) JSSK- pregnant women and infants till 1 year of age

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

Q131. Narrowest part of middle ear is :
a) Hypo tympanum
b) Epitympanum
c) Attic
d) Mesotympanum

Q132. Cerebrospinal rhinorrhea is usually due to fracture of :
a) Sphenoid bone
b) Cribriform plate
c) Roof of orbit
d) Petrous part of temporal bone

Q133. Inferior turbinate is a part of :
a) Maxilla
b) Sphenoid
c) Ethmoid
d) Separate bone

Q134. Regarding ethmoidal polyp , which one of the following is true ?
a) Unilateral
b) Epistaxis
c) Less than 10 yr age
d) Asso. To bronchial asthma

Q135. B type tympanogram is seen in :
a) Serous otitis media
b) Ossicular discontinuity
c) Otosclerosis
d) All of the above

Q136. Myringotomy in ASOM is done in which quadrant ?
a) Anteriosuperior
b) Posteriosuperior

c) Anterioinferior
d) Posterioinferior

Q137. Etiology of pulsatile tinnitus includes the following except :
a) AV malformation of neck
b) Otosclerosis
c) Glomus juglare tumor
d) Hyperthyroidism

Q138. Most developed paranasal sinus at birth :
a) Frontal
b) Maxillary
c) Sphenoid
d) Ethmoid

Q139. Most reliable test for Eustachian tube dysfunction :
a) Politzer test
b) Tympanometry
c) Rhinomanometry
d) Vemp

Q140. Schwartz operation is also called as :
a) Modified radical mastoidectomy
b) Radical mastoidectomy
c) Bondys mastoidectomy
d) Cortical mastoidectomy

Q141. Occulomoter nerve palsy affects all of the following muscles, except:
a) Medial rectus
b) Inferior oblique
c) Lateral rectus
d) Levetorpalpabraesuperioris

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

Q142. 63-year-old man presents with dysphagia and halitosis. He regurgitates undigested food hours after meals. Barium swallow shows a posterior hypopharyngeal pouch above cricopharyngeus.

Site of herniation?

a) Triangle of Calot
b) Killian dehiscence
c) Hesselbach triangle
d) Foramen of Winslow

Q143. A 54-year-old obese male has chronic GERD symptoms for 15 years. Endoscopy reveals salmon-colored mucosa extending 5 cm above the gastroesophageal junction. The major concern in this patient is development of:

a) Squamous cell carcinoma
b) Adenocarcinoma of esophagus
c) GIST
d) Leiomyoma

Q144. A primigravida at 33 weeks of gestation was rushed to the hospital with recurrent seizures. Her husband says that prior to the seizures, she had complained of headache and visual disturbances. She was on antihypertensives from the first trimester and there is no history of neurological disease. Which of the following is true regarding her management?

a) First step of management is intravenous $MgSO_4$
b) Diazepam is recommended as an anticonvulsant if $MgSO_4$ fails
c) First step is to maintain airway
d) Maternal alkalosis commonly occurs and needs correction

Q145. A 23-year-old sexually active woman presents with lower abdominal pain, fever, and vaginal discharge for 4 days. On pelvic examination, cervical motion tenderness and adnexal tenderness are noted. What is most likely diagnosis ?

a) Endometriosis
b) Adenomyosis
c) Lower abdominal pain syndrome
d) Endometrial Polyp

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

Q146. A woman develops atonic PPH after vaginal delivery. Oxytocin, methylergometrine, carboprost, and uterine massage fail. Bakri balloon inserted but bleeding persists. She is hemodynamically stable and desires future fertility. Best next step?

a) Internal iliac artery ligation
b) B-Lynch suture
c) Hysterectomy
d) Tranexamic acid infusion

Q147. When a child is not able to perform the following motor functions such as going upstairs and downstairs alternate foot per step or hopping in place, his motor development is considered below:

a) 2 years
b) 3 years
c) 4 years
d) 5 years

Q148. A 8 yrs old patient presented with Upper Humerus Lytic lesion with a cortical fragment at the bottom of the cavity. All are treatment modalities except :

a) Radiotherapy
b) Injection of a Sclerosant
c) Curettage and Bone grafting
d) Intralesional Steroid injection

Q149. A 5-year-old child is brought to the dermatology OPD with multiple itchy skin lesions around the mouth and nose.
The lesions initially appeared as small red papules and vesicles.
Over the next few days, they became pustular and ruptured.
On examination, multiple erosions with thick honey-colored crusts are seen over the perioral and perinasal region.
There are no large flaccid bullae or generalized peeling of skin.
The child is otherwise stable and afebrile.
The pediatrician explains that this condition may rarely be followed by a renal complication.
Which of the following is the correct statement?

MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

a) Bullous impetigo – Streptococcus
b) Non-bullous impetigo – Staphylococcus only
c) Bullous impetigo – Staphylococcal scalded skin syndrome
d) Non-bullous impetigo – Post-streptococcal glomerulonephritis

Q150. A new anesthesia resident is posted in the operation theatre complex of a tertiary-care hospital. During orientation, the biomedical engineer shows a large white insulated tank installed outside the hospital.
It supplies oxygen to the central medical gas pipeline system for ICUs and OTs.
The tank contains oxygen in liquid form and converts it to gas before delivery through the pipeline.
It has vacuum insulation to reduce heat entry and evaporation.
The resident is asked to identify the equipment and choose the incorrect statement about it.
Which of the following is not true?

a) It stores liquid oxygen
b) It stores oxygen at temperature 0°C
c) It is called Vacuum Insulated Evaporator
d) Oxygen is stored at lower pressure than in a cylinder

ANSWERS

1	b		5	b		9	b		13	c		17	d
2	c		6	b		10	c		14	a		18	d
3	b		7	b		11	b		15	b		19	d
4	a		8	b		12	b		16	d		20	d

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MOCK TEST – 5 PAPER -1 DATED ON 13th JUNE 2026

21	d	47	d	73	d	99	c	125	c
22	d	48	d	74	c	100	c	126	b
23	d	49	a	75	d	101	c	127	b
24	d	50	a	76	d	102	b	128	b
25	c	51	d	77	c	103	c	129	c
26	d	52	c	78	d	104	b	130	b
27	d	53	a	79	c	105	c	131	d
28	d	54	c	80	b	106	b	132	b
29	d	55	b	81	b	107	c	133	d
30	d	56	c	82	b	108	b	134	d
31	d	57	d	83	b	109	b	135	a
32	d	58	c	84	b	110	a	136	d
33	d	59	c	85	b	111	a	137	b
34	d	60	b	86	b	112	b	138	d
35	b	61	b	87	b	113	b	139	b
36	c	62	a	88	c	114	c	140	d
37	c	63	b	89	b	115	b	141	c
38	a	64	b	90	c	116	d	142	b
39	c	65	c	91	d	117	b	143	b
40	b	66	d	92	b	118	b	144	c
41	c	67	b	93	b	119	b	145	c
42	c	68	d	94	b	120	c	146	b
43	c	69	c	95	b	121	c	147	c
44	d	70	d	96	a	122	d	148	a
45	c	71	d	97	d	123	b	149	d
46	a	72	c	98	a	124	a	150	b